BLUEPRINT FOR CHANGE

IMPROVE ROAD DESIGN: Regular safety audits of all national and state highways must identify and rectify blackspots. States must ensure all clearances have been taken at the design stage itself

MONITOR VEHICLES BETTER: States must implement Vehicle Scrapping Policy, 2021, to ensure that old and unfit vehicles are off the roads. Manufacturers, too, must provide safety enhancement features across models, not just in luxury segment

STRICTER ENFORCEMENT OF TRAFFIC RULES: No concession must be given to traffic violations, strict penalties must be enforced for deterrence

TOUGHER LICENCE NORMS: States must set up driving schools and test centres, clearing which should be made mandatory for a person to get a driving licence

POST-ACCIDENT MEASURES: Set up a universal helpline number for accident victims; improve response time of ambulances and ensure they have ventilators and cutting tools

FORTIFY TRAUMA CARE CENTRE NETWORK: Ensure the several commissionaires of toll roads set up ambulance and medical facilities as mandated; and all hospitals are equipped for emergency health

SPREAD AWARENESS: Foster compliance with traffic regulations by incorporating road safety education into school programmes and continuing public awareness campaigns

ommends a response time of 20 minutes for urban areas and 30 minutes for rural areas, and under 10 minutes for cardiac, respiratory and stroke cases. A SaveLIFE audit of 21 highways in 16 states under its zero fatality corridor project found the average response time of ambulances to be 28.7 minutes.

"In most cases," says Tewari, "patients are dumped into an ambulance and taken to hospital with little care given in transit." Sometimes, the paramedics even fail to correlate the nature of injuries to the type of facility the patient must be taken to. "The referral network after an emergency call must be made clear," adds Tewari. "There should also be a notification system for an ambulance to inform the facility it is headed to in advance." A NITI Aayog study found this pre-hospital arrival notification system missing at most facilities. Another problem is that if there's an accident involving a car or a bus and the vehicle is in such a position that it needs to be cut open, there is no equipment available for that. "So, we have decided to change the ambulance code," says Gadkari. The new code will ensure that ambulances are equipped with engineering tools besides ventilators, and a trained compounder.

NEED FOR MORE TRAUMA CARE CENTRES

In 2007, a national programme was launched to construct a pan-India trauma care network in government hospitals to ensure that no road crash victim has to be transported for more than 50 km, for which a trauma centre was to be designated every 100 km. Even as 196 such facilities were sanctioned at district hospitals and medical colleges near national and state highways over the next decade, no provision was made to identify and fund new trauma centres after 2017. "With the privatisation of roads, and the NHAI leasing out stretches of toll roads, there are now multiple concessionaires who are mandated to have an ambulance and medical facilities on their respective stretches," says Ramesh Kumar. "But there is hardly any monitoring to ensure compliance."

The 'trauma' doesn't end there. For those who do reach hospital on time, they cannot be sure of proper care. "There is a huge discrepancy in the quality and availability of emergency care equipment in the country," says Dr Venkat Raman Kola, clinical director, Yashoda Hospitals, Hyderabad. Surveys have showed that critical life-saving equipment was not available in 45-60 per cent of the hospitals while 50 per cent didn't have an inhouse blood bank. Many experts feel the need for minimum standards and protocols laid out through an enforceable legislative act for emergency response and trauma care in India. Australia, Brazil, Germany, Malaysia, South Africa, the UK, the US, and even Pakistan have written statutes for a guaranteed right to emergency health.

While India awaits such a law, there have been some positive developments on other fronts. The Good Samaritan law, for instance. Bystander care is of paramount importance in road crash cases. "With the clock ticking," says Dr Kola, "if no one comes forward to help a trauma victim, the chances of treatment making a difference will plummet." In its landmark judgment in 2016, the Supreme Court incorporated the guidelines for the protection of Good Samaritans—those who voluntarily come forward to administer immediate assistance or emergency care to a person injured in an accident. The Centre has also provided them protection under the amended Motor Vehicles Act in 2019. The new law protects Good Samaritans from any harassment or civil and criminal proceedings on account of actions they have taken to save the life of a crash victim. A Good Samaritan Scheme was launched in 2021 to reward those who came to the rescue of accident victims.

The Union and state governments along with the private sector and NGOs have to work together to overcome all challenges to safety on highways and ensure India sheds the ignominy of being among the countries that have the highest rate of road accidents. The government, on its part, is taking accountability very seriously. "We don't want to tell people it is not our fault; we accept responsibility," says Gadkari. That acknowledgment itself is a welcome first step to tackle our foremost public health hazard. ■